

NIC CPD Micro-Credential: Nutrition, Hydration & Mealtime Support for Professional Caregivers

Issuing organisation: National Institute of Caregivers Nigeria (NIC)

Credential type: Stand-alone Continuing Professional Development (CPD) Micro-Credential

Recommended duration: 5–6 CPD learning hours

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Scope note: This is professional development for caregivers and direct-care personnel. It is not a dietetics qualification, clinical nutrition qualification, speech and language therapy qualification, dysphagia-assessment qualification, first-aid replacement or clinical licence. It does not authorise a learner to prescribe diets, determine individual nutritional requirements, diagnose swallowing disorders, independently modify food or fluid consistency, or replace an authorised care plan.

1. Course title

NIC CPD Micro-Credential: Nutrition, Hydration & Mealtime Support for Professional Caregivers

2. Course overview

This CPD strengthens the caregiver's ability to support safe, dignified and person-centred eating and drinking within an authorised role. It focuses on nutrition awareness, hydration, mealtime support, appetite, food preferences, cultural considerations, independence, malnutrition risk, dehydration risk, swallowing awareness, choking risk, positioning, environmental factors, food safety, allergies, observation, documentation and escalation.

Mealtimes are not merely a task of getting food into a client. They may provide nutrition, hydration, comfort, pleasure, social connection, cultural identity, routine, independence and dignity. The course therefore uses the principle:

Safe nutrition is not only about what a person eats. It is also about how, when, where and with whom they are supported.

The central professional sequence is:

KNOW THE PERSON → PREPARE SAFELY → SUPPORT CHOICE & INDEPENDENCE → OBSERVE → RECOGNISE RISK → RESPOND WITHIN SCOPE → DOCUMENT → REPORT / ESCALATE.

The course assumes that learners already understand basic caregiving, dignity, person-centred care, communication, boundaries, documentation and reporting. It complements NIC's full caregiver programmes rather than reproducing introductory content.

Course at a glance

Element	Specification
Target learners	Professional caregivers, healthcare assistants, home-care workers, residential-care staff, care supervisors, community-care workers and other direct-care personnel
Course type	Stand-alone CPD micro-credential
Learning time	5–6 CPD hours
Structure	6 modules, 18 lessons, module assessments and a 35-question final assessment
Main focus	Safe, dignified and person-centred eating and drinking within caregiver scope
Recommended final pass mark	80% (28/35)
Application focus	At least 25 of 35 final questions are scenario/application based
Practical limitation	Online completion demonstrates learning and scenario reasoning; it does not prove real-world feeding or choking-response competence
Separate training	Hands-on first-aid certification is recommended where emergency competence is required

3. Strategic purpose of the CPD

Eating and drinking are routine parts of care, but they involve multiple safety and dignity risks. A caregiver may need to support a person who eats very little, drinks only with prompting, loses weight, coughs during fluids, refuses a familiar meal, tires while eating,

has a known allergy or has an authorised modified-texture plan. A hurried or improvised response can cause malnutrition, dehydration, aspiration, choking, allergic reaction, foodborne illness, loss of independence or avoidable distress.

This CPD develops observation and judgement rather than diet prescription. It teaches caregivers to recognise warning signs, support safe intake, preserve choice and independence, follow authorised instructions, avoid unsafe improvisation and escalate meaningful changes. Nutrition awareness is distinguished from individual clinical dietary management. A caregiver may notice that a client is eating less; an authorised clinician or nutrition professional assesses the cause and determines treatment.

The course is relevant to Nigerian homes, residential care, community services and healthcare-support environments. It recognises foods, family routines, religious practices, language diversity, hospitality and multigenerational households without prescribing one “Nigerian diet” or assuming that all clients have the same preferences. Current local policies, emergency contacts, food-safety requirements and authorised texture plans must be added by NIC and employers before publication.

4. Target audience

The course is intended for professional caregivers, healthcare assistants, home-care workers, residential-care staff, care supervisors, community-care workers and other direct-care personnel who support clients with eating, drinking and mealtime routines within an authorised role.

It is not intended to train dietitians, nutritionists, speech and language therapists, dysphagia specialists, clinicians or first-aid instructors. Learners who lack basic caregiving knowledge should complete foundational education first.

5. Recommended CPD hours

NIC should publish **5–6 CPD learning hours**. A practical allocation is approximately 3.5–4 hours for lessons, 60–75 minutes for lesson and module assessments, and 45–60 minutes for the final assessment, review and completion record. Credit should reflect meaningful interaction and assessment, not merely time spent with a page open.

6. Prerequisites

The recommended prerequisite is prior caregiving knowledge or experience, including basic dignity, communication, observation, documentation, professional boundaries and escalation. NIC may accept a foundational caregiver programme, Elderly Care &

Gerontology programme, another recognised caregiver course or evidence of current direct-care work.

7. Learning outcomes

By the end of the CPD, the learner should be able to:

1. **Explain** basic nutrition concepts relevant to caregiving without prescribing individual diets.
2. **Support** person-centred mealtimes that respect preferences, culture, religion, communication needs, dignity and independence.
3. **Recognise** possible hydration concerns and factors that increase dehydration risk.
4. **Recognise** possible malnutrition warning signs without independently diagnosing malnutrition.
5. **Respond** appropriately to poor appetite, food refusal, slow eating and meaningful changes in intake.
6. **Identify** possible swallowing concerns, aspiration warning signs and choking risk.
7. **Follow** authorised texture-modified, thickened-fluid, allergy and dietary-restriction instructions without improvising.
8. **Apply** caregiver-level positioning, environment, food-safety and mealtime-support principles.
9. **Document** intake patterns, observations, risks, responses and escalation objectively.
10. **Escalate** urgent or meaningful nutrition, hydration, allergy, choking, swallowing and safety concerns through the correct pathway.

8. Competency framework

Domain	Expected capability	Evidence
Nutrition awareness	Understands basic nutrients and the relationship between adequate intake, strength, recovery and wellbeing	Module 1
Hydration	Recognises reduced intake and possible dehydration risk without diagnosing	Module 2
Person-centred mealtimes	Supports choice, culture, pleasure, dignity and	Module 3

	communication	
Independence	Uses support rather than substitution and allows adequate time	Modules 3 and 5
Malnutrition awareness	Notifies weight loss, poor appetite, weakness and missed meals and escalates	Module 4
Swallowing safety	Recognises possible dysphagia and aspiration signs and stops unsafe support	Module 4
Choking awareness	Distinguishes coughing, choking and severe obstruction and follows trained emergency procedure	Module 5
Food safety	Applies clean preparation, storage, separation, reheating and expiry awareness	Module 5
Allergies and restrictions	Checks authorised information, prevents cross-contact and escalates reactions	Module 6
Documentation and escalation	Records facts and reports concerns without diagnosis or unsafe improvisation	Module 6

9. Nutrition & hydration response framework

The NIC KNOW–CARE sequence

K — Know the person. Learn preferences, culture, religious practice, communication needs, usual intake, independence, allergies, restrictions and authorised plans.

N — Notice the plan. Check the current care plan, texture or fluid instructions, allergy information, positioning requirements and escalation contacts before the meal.

O — Organise safely. Prepare clean food and equipment, a safe environment, suitable seating and any authorised adaptive aids.

W — Welcome choice. Explain, offer reasonable options, support independence and preserve dignity.

C — Carefully observe. Notice intake, fatigue, coughing, wet voice, food remaining in the mouth, refusal, distress, pain and changes from usual pattern.

A — Act within scope. Provide only authorised support. Stop if support appears unsafe. Do not prescribe, diagnose, alter consistency or improvise substitutions.

R — Record meaningfully. Document what was offered, taken or refused, relevant signs, support given, outcome and who was informed.

E — Escalate appropriately. Report urgent choking, allergic reaction, aspiration concern, sudden deterioration, repeated low intake, suspected dehydration, weight loss or unavailable authorised food/fluid through the correct route.

When in doubt: stop unsafe support, protect the person, follow the authorised plan and seek help.

10. Complete module structure

Module	Title	Lessons	Assessment
1	Nutrition Foundations for Caregivers	1. Nutrition Awareness; 2. Balanced Meals and Adequate Intake; 3. Nutrition Scope and Individual Needs	6 questions
2	Hydration and Fluid Support	4. Why Fluids Matter; 5. Recognising Hydration Risk; 6. Supporting Fluids and Escalating Concern	6 questions
3	Person-Centred Mealtimes and Independence	7. Preferences, Culture and Dignity; 8. Appetite and Food Refusal; 9. Environment, Communication and Independence	6 questions
4	Malnutrition and Swallowing Awareness	10. Malnutrition Risk; 11. Swallowing and Aspiration Awareness; 12. Texture-Modified Diets and Safe Escalation	7 questions

5	Choking, Positioning and Food Safety	13. Safe Positioning and Mealtime Attention; 14. Choking Awareness and Emergency Response; 15. Food Safety in Care Settings	7 questions
6	Allergies, Restrictions, Documentation and Escalation	16. Allergies, Intolerances and Dietary Restrictions; 17. Documentation and Handover; 18. Integrated Mealtime Risk Judgement	7 questions

11. Detailed lesson outline

Lesson	Purpose	Core application
1	Build caregiver-level nutrition awareness	Connect adequate intake to strength, recovery and wellbeing
2	Understand balanced meals without prescribing	Recognise broad nutrient roles and variety
3	Keep within scope	Distinguish awareness from clinical dietary management
4	Understand hydration	Support fluids through accessible, preferred options
5	Recognise hydration risk	Notice reduced intake and associated concerns
6	Escalate hydration concerns	Observe, record and report without prescribing volumes
7	Person-centred mealtimes	Respect food, culture, faith, routine and sensory preference
8	Appetite and food refusal	Explore contributors without force
9	Environment and independence	Adapt seating, pace, aids and communication

10	Malnutrition awareness	Notice weight loss, weakness and missed meals
11	Swallowing awareness	Recognise possible dysphagia and aspiration signs
12	Texture-modified plans	Follow authorised instructions and avoid improvisation
13	Positioning and attention	Support stable, upright, calm and dignified meals
14	Choking awareness	Follow trained first-aid and emergency procedures
15	Food safety	Reduce contamination, storage and expiry risks
16	Allergies and restrictions	Check, prevent cross-contact and escalate reactions
17	Documentation and handover	Record meaningful facts and communicate risk
18	Integrated judgement	Apply KNOW-CARE to complex mealtime situations

12. Full lesson content

Module 1 — Nutrition Foundations for Caregivers

Lesson 1. Nutrition Awareness

Why this matters in professional caregiving

Food provides energy and building materials needed for daily activity, strength, recovery, wound healing, immune function, mobility and general wellbeing. A client who eats very little may become weaker, tire more easily, lose interest in activity or struggle to recover from illness. Caregivers are often best placed to notice changes in appetite, portions, routines and ability to access food.

Learning objectives

The learner should be able to describe the caregiver's nutrition role; explain why adequate intake matters; identify changes worth documenting; and distinguish observation from dietary diagnosis.

Core instructional content

Nutrition awareness means recognising whether a person is receiving and tolerating enough food for their individual care plan, preferences and health needs. It does not require the caregiver to calculate calories or prescribe a diet. Caregivers should know what the authorised plan says, support access to food, notice patterns and report concerns.

Adequate food intake supports energy for movement and participation. Protein-containing foods contribute to maintenance and repair of body tissues. Carbohydrate-containing foods provide energy. Fats provide energy and help the body use certain vitamins. Vitamins and minerals support many body functions. Fibre and fluids support bowel regularity, although individual needs and restrictions vary. This is general awareness, not individual treatment.

A balanced meal is not a fixed menu. It may involve a suitable combination of foods available to the client, taking account of preferences, culture, religion, allergy, texture, health conditions and authorised advice. Do not assume that one food list or portion is right for everyone.

Applying This in Practice

In Nigeria, meals may include staples, legumes, vegetables, fruits, fish, eggs, meat, dairy or locally available alternatives. Examples should demonstrate variety and choice, not prescribe a single national diet. A caregiver should follow the client's plan and ask the responsible professional when an authorised diet is unclear.

Case scenario

A client who normally eats most of lunch takes only two spoonfuls for three consecutive days and appears weaker.

Correct professional response: Notice the change, observe possible contributors, document intake and associated signs, and report for appropriate assessment. Do not independently prescribe supplements or force food.

Common risks or mistakes

Risks include reducing nutrition to calories, forcing food, assuming refusal is stubbornness, prescribing supplements, ignoring culturally important foods and failing to report repeated low intake.

Key practice points

1. Nutrition supports energy, strength, recovery and wellbeing.
2. Caregivers observe and support; they do not prescribe.
3. Patterns matter more than one isolated meal.
4. Food must be suitable for the authorised plan.
5. Document and report meaningful change.

Lesson assessment

1. **MCQ:** The caregiver's nutrition role is to: A) prescribe a diet; B) support the authorised plan, observe intake and report concerns; C) calculate all nutrient requirements; D) diagnose malnutrition. **Answer: B. Objective tested: 1.**
2. **Scenario:** A client eats much less than usual for three days. **Answer:** Document, observe and report. **Objective tested: 3.**
3. **True/False:** One standard meal plan suits every client. **Answer: False. Objective tested: 2.**
4. **What should the caregiver NOT do?** Force a client to finish a meal. **Answer:** Force undermines dignity and may create safety risks. **Objective tested: 2.**
5. **Application:** Name three benefits of adequate nutrition. **Answer:** Energy, strength, recovery, wound healing, immunity, mobility or wellbeing. **Objective tested: 1.**
6. **Scope question:** Who determines individual clinical nutrition therapy? **Answer:** An appropriately authorised nutrition or clinical professional. **Objective tested: 1 and 3.**

Lesson 2. Balanced Meals and Adequate Intake

Why this matters in professional caregiving

Caregivers do not need dietetics-level knowledge, but they need enough awareness to notice when a meal lacks variety, when a client cannot access food or when a person's needs are not being met. Broad knowledge helps the caregiver communicate accurately and avoid misleading claims.

Learning objectives

The learner should be able to describe macronutrients, fibre, vitamins and minerals at a basic level; recognise the value of variety; and support food access without prescribing.

Core instructional content

Macronutrients are nutrients needed in larger amounts. Carbohydrates commonly provide energy; protein supports tissue maintenance and repair; fats provide energy and contribute to body functions. Vitamins and minerals are needed in smaller amounts for many processes. Fibre supports bowel function for many people, and fluids are essential to hydration. The caregiver should not turn these categories into a rigid list of “good” and “bad” foods.

Adequate intake depends on the individual. Age, activity, illness, appetite, swallowing ability, dental health, medications, cultural practice, income, access, care plan and clinical conditions may affect food choices and amounts. A caregiver should not infer requirements from appearance or offer high-protein, low-salt, diabetic, renal or weight-loss diets without authorised instruction.

Support may involve preparing the eating space, opening packages, arranging food, offering authorised snacks, assisting with shopping under policy, recording what was taken and reporting barriers. Food should be appetising, appropriately presented and served at a safe temperature according to the plan and food-safety procedures.

Applying This in Practice

Use familiar foods and locally available options that fit the person’s preferences and authorised restrictions. A client may prefer smaller portions more often, certain textures or particular meal times. Ask, observe and report rather than imposing a standard.

Case scenario

A caregiver decides that a client needs a high-protein diet and begins adding powdered supplements without consultation.

Correct professional response: Stop the unauthorised change, check the plan and ask the responsible professional for guidance. Record the concern and do not present a personal diet decision as treatment.

Common risks or mistakes

Risks include diet prescription, supplement use without authorisation, moralising foods, ignoring access barriers, using large portions that overwhelm the client and assuming “healthy” means suitable for every condition.

Key practice points

1. Know broad nutrition concepts but remain within role.
2. Variety and adequacy are general principles, not individual prescriptions.
3. Follow authorised restrictions and care plans.

4. Support access, presentation and choice.
5. Report persistent barriers or reduced intake.

Lesson assessment

1. **MCQ:** Protein is broadly associated with: A) tissue maintenance and repair; B) a reason to prescribe supplements; C) a cure for all weakness; D) a replacement for fluids.
Answer: A. Objective tested: 1.
2. **Scenario:** A client needs smaller, more frequent meals according to an authorised plan.
Answer: Follow the plan and document intake. **Objective tested: 2–3.**
3. **True/False:** “Healthy” food is automatically suitable for every client. **Answer: False. Objective tested: 2.**
4. **What should the caregiver NOT do?** Add supplements independently. **Answer:** Individual nutrition therapy requires authorisation. **Objective tested: 3.**
5. **Application:** Name two barriers to adequate food intake. **Answer:** Poor appetite, cost, access, pain, fatigue, swallowing difficulty, dental problems or inability to prepare food. **Objective tested: 2–4.**
6. **Best next action:** The client cannot open the food package. **Answer:** Assist within role or arrange appropriate help while preserving independence. **Objective tested: 4.**

Lesson 3. Nutrition Scope and Individual Needs

Why this matters in professional caregiving

The boundary between general nutrition awareness and clinical dietary management protects clients. A caregiver may be asked to recommend a diabetic diet, alter salt, add supplements, calculate fluids or exclude foods. Good intentions do not make an unauthorised plan safe.

Learning objectives

The learner should be able to distinguish general nutrition awareness from clinical management; identify requests outside role; and follow authorised instructions without improvisation.

Core instructional content

General nutrition awareness includes supporting food access, respecting preferences, observing intake, identifying barriers and reporting changes. Clinical dietary management may include therapeutic diets, prescribed supplements, texture modifications, fluid restrictions, enteral feeding, allergy management, renal or cardiac dietary adjustments and

individual nutrient or fluid targets. These require appropriate assessment and authorisation.

If a plan is missing, contradictory or impossible to implement, pause the unsafe part and contact the responsible professional. Do not choose a replacement based on internet advice, family instruction or personal experience. If a food or fluid is unavailable, follow the escalation route rather than improvising.

The caregiver should explain boundaries respectfully: “I can support the plan and record what happens, but I cannot change the diet. I will contact the responsible person.” This protects the client and maintains professional credibility.

Applying This in Practice

In home care, relatives may offer advice based on another person’s illness. In residential care, staff may say a substitute is “close enough.” In all settings, use the current authorised plan and document barriers.

Case scenario

A family member asks the caregiver to stop all salt because the client’s blood pressure was high last week, but no updated plan exists.

Correct professional response: Do not independently prescribe a restriction. Continue the authorised plan, document the request and seek guidance from the responsible professional.

Common risks or mistakes

Risks include internet-led prescribing, family-led treatment changes, supplement use, changing fluid targets, making substitutions and assuming one client’s diet applies to another.

Key practice points

1. Awareness is not clinical management.
2. Never prescribe or alter a therapeutic plan.
3. Stop unsafe improvisation and seek authorised guidance.
4. Explain scope without dismissing the concern.
5. Document missing or unworkable instructions.

Lesson assessment

1. **MCQ:** Which is outside ordinary caregiver scope? A) recording intake; B) supporting an authorised meal; C) prescribing a renal diet; D) opening a package. **Answer: C.**
Objective tested: 1.
2. **Scenario:** The texture instruction is unclear. **Answer:** Stop the uncertain support and seek authorised clarification. **Objective tested: 2–3.**
3. **True/False:** A family member can change the client's therapeutic diet verbally. **Answer: False. Objective tested: 2.**
4. **What should the caregiver NOT do?** Use an internet diet as a substitute plan. **Answer:** Seek qualified guidance. **Objective tested: 2–3.**
5. **Application:** Give one respectful scope statement. **Answer:** "I can support the authorised plan, but I cannot change the diet; I will seek guidance." **Objective tested: 3.**
6. **Documentation question:** Record the request, current plan, action taken and person contacted. **Answer:** Correct. **Objective tested: 3.**

Module 2 — Hydration and Fluid Support

Lesson 4. Why Fluids Matter

Why this matters in professional caregiving

Fluids support body functions, comfort and daily wellbeing. Some clients depend on others to make drinks accessible, remember to offer them, open containers or position them safely. Hot environments, illness, vomiting, diarrhoea, mobility limitations, swallowing difficulty and reduced alertness can increase risk.

Learning objectives

The learner should be able to explain why fluids matter; identify common sources; recognise access barriers; and support drinks within the authorised plan.

Core instructional content

Fluids may come from water, other suitable drinks, soups, fruits and foods with high moisture content, depending on the person's plan. Do not assume that only plain water counts. Preferences, culture, religion, temperature, taste, dental status, swallowing instructions, medical restrictions and access all matter.

Older adults and people with mobility limitations may rely on another person to bring fluids. A person may not ask because of embarrassment, fatigue, communication difficulty

or fear of incontinence. A safe caregiver makes drinks accessible in the authorised way, offers them at appropriate times and records concerns.

Some clients have fluid restrictions or thickened-fluid instructions. Never set an individual target or change consistency without authorisation. Encourage only what is permitted by the plan and escalate if the person cannot take fluids safely.

Applying This in Practice

In hot Nigerian environments, power outages or long journeys may affect access to safe, preferred drinks. Plan within the care setting's procedures, use safe water and keep suitable drinks accessible without creating spills or choking risk.

Case scenario

A client drinks very little because the cup is difficult to hold and the caregiver offers only at mealtimes.

Correct professional response: Provide an authorised suitable cup or adaptive aid, offer fluids at appropriate intervals according to the plan, preserve independence and document reduced intake.

Common risks or mistakes

Risks include forcing fluids, ignoring restrictions, offering unsafe textures, using dirty containers, failing to offer regularly and assuming lack of request means lack of need.

Key practice points

1. Fluids can come from drinks and suitable foods.
2. Access and dependence affect hydration risk.
3. Follow fluid restrictions and texture instructions exactly.
4. Offer and observe; do not force.
5. Record reduced intake and barriers.

Lesson assessment

1. **MCQ:** Hydration support includes: A) making authorised fluids accessible; B) prescribing a fluid target; C) ignoring preferences; D) forcing drinks. **Answer: A. Objective tested: 1–2.**
2. **Scenario:** A cup is difficult to hold. **Answer:** Use an authorised adaptive aid and observe intake. **Objective tested: 3.**

3. **True/False:** A person who does not ask for a drink cannot be dehydrated. **Answer:** False. **Objective tested:** 2.
4. **What should the caregiver NOT do?** Change a thickened-fluid instruction. **Answer:** Follow the authorised plan and escalate. **Objective tested:** 3.
5. **Application:** Name three factors increasing hydration risk. **Answer:** Heat, illness, vomiting/diarrhoea, mobility limitation, reduced alertness, dependence or swallowing difficulty. **Objective tested:** 2.
6. **Documentation question:** Record offered fluid, amount taken where required, refusal/barrier and person informed. **Answer:** Correct. **Objective tested:** 4–5.

Lesson 5. Recognising Hydration Risk

Why this matters in professional caregiving

Reduced intake can be easy to miss, especially when a person has memory, communication, mobility or swallowing difficulties. Caregivers should notice possible warning signs while understanding that no single sign proves dehydration. The correct sequence is **OBSERVE → RECORD → REPORT**.

Learning objectives

The learner should be able to identify possible hydration concerns; avoid overdiagnosis; identify contributing factors; and escalate meaningful changes.

Core instructional content

Possible observable concerns include dry mouth, unusual thirst, reduced urination where observable, dark urine where relevant, dizziness, weakness, fatigue, headache or confusion. These signs can have other causes and should not be treated as definitive proof. A caregiver should report patterns, associated symptoms and changes from usual.

Risk can increase with hot weather, fever, vomiting, diarrhoea, reduced consciousness, dependence on others, mobility limitation, swallowing difficulty, pain, nausea, limited access, fear of incontinence or a fluid restriction that is not being implemented correctly.

Do not independently prescribe fluid volumes, stop a restriction, give oral rehydration products or tell a client to drink beyond an authorised plan. If a person cannot swallow safely, stop unsafe support and seek guidance.

Applying This in Practice

Use a secure food-and-fluid observation record. Include time, drink offered, amount taken where required by policy, refusal, symptoms, environmental conditions, support provided

and escalation. In hot settings, report repeated low intake early.

Case scenario

A client is unusually weak, has a dry mouth and has taken very little fluid. A relative tells the caregiver to give several large cups quickly.

Correct professional response: Do not force or rapidly give fluids. Keep the person safe, follow the authorised plan, document the signs and escalate for appropriate assessment.

Common risks or mistakes

Risks include diagnosing dehydration from one sign, forcing drinks, ignoring fluid restrictions, delaying escalation and failing to record amounts or symptoms.

Key practice points

1. Signs are possible indicators, not proof.
2. Observe the pattern and context.
3. Do not prescribe volumes or override restrictions.
4. Stop if swallowing appears unsafe.
5. Report meaningful or urgent concerns promptly.

Lesson assessment

1. **MCQ:** A dry mouth is: A) definitive proof; B) a possible concern requiring context and observation; C) irrelevant; D) a reason to force fluids. **Answer: B. Objective tested: 2.**
2. **Scenario:** A client is weak and has taken little fluid. **Answer:** Document, follow the plan and escalate. **Objective tested: 3–5.**
3. **True/False:** The caregiver may prescribe a fluid volume. **Answer: False. Objective tested: 3.**
4. **What should the caregiver NOT do?** Give several large drinks rapidly to “catch up.” **Answer:** It may be unsafe and ignores individual instructions. **Objective tested: 4.**
5. **Application:** Name three signs or risk factors. **Answer:** Dry mouth, thirst, reduced urination, dark urine, dizziness, weakness, heat, illness or dependence. **Objective tested: 2.**
6. **Documentation question:** Why record context? **Answer:** Signs have multiple possible causes and context supports appropriate assessment. **Objective tested: 2–5.**

Lesson 6. Supporting Fluids and Escalating Concern

Why this matters in professional caregiving

The caregiver must balance encouragement with safety. A client may drink little because of preference, pain, nausea, swallowing difficulty, fear of incontinence or poor access. Repeated prompting without investigating barriers can increase distress and does not solve the problem.

Learning objectives

The learner should be able to encourage fluids respectfully; adapt access within role; recognise unsafe swallowing; and report low intake, vomiting, diarrhoea or sudden change.

Core instructional content

Offer suitable drinks in a clean, accessible container and at an appropriate temperature. Use the person's preferences and authorised plan. Offer at suitable points during the day rather than waiting for thirst. Explain and ask; do not force. If the person refuses, ask what is wrong and record the response.

If a person coughs repeatedly, develops a wet voice or appears to struggle with fluids, stop unsafe support and follow the swallowing escalation route. Do not independently thicken the drink, dilute it, switch to a different product or continue testing.

Vomiting and diarrhoea can increase fluid risk. Record frequency and associated concerns within role, provide safe comfort and escalate according to the care plan. Sudden confusion, weakness, fainting or inability to keep fluids down may require urgent assessment.

Applying This in Practice

At home, keep approved drinks within reach but avoid spills, contaminated containers or unsafe placement. In residential care, communicate across shifts. During travel or power outages, plan safe water access and report supply problems early.

Case scenario

A client coughs each time they drink thin fluid. A family member suggests adding flour to thicken it.

Correct professional response: Stop the unsafe fluid support, follow the existing plan if one exists and contact the authorised professional. Do not improvise thickening.

Common risks or mistakes

Risks include force, home-made thickener, ignoring coughing, continuing to “test,” changing restrictions and failing to report repeated low intake.

Key practice points

1. Encourage respectfully and offer accessible preferred fluids.
2. Refusal should be explored, not punished.
3. Coughing or wet voice may signal swallowing concern.
4. Stop unsafe support and follow the plan.
5. Report vomiting, diarrhoea, low intake or acute change.

Lesson assessment

1. **Scenario:** A client refuses a drink. **Answer:** Ask the reason, offer an appropriate alternative if authorised and document/report repeated refusal. **Objective tested: 4.**
2. **MCQ:** Repeated coughing with fluids requires: A) continued testing; B) stop unsafe support and escalate; C) flour; D) force. **Answer: B. Objective tested: 6 and 10.**
3. **True/False:** A caregiver may thicken fluids with a household ingredient. **Answer: False. Objective tested: 7.**
4. **What should the caregiver NOT do?** Change a fluid restriction independently. **Answer:** Seek authorised guidance. **Objective tested: 3.**
5. **Application:** Name two ways to encourage fluids. **Answer:** Use a preferred approved drink, accessible cup, suitable timing, calm prompting or social support. **Objective tested: 6.**
6. **Scenario:** Vomiting continues and the client is weak. **Answer:** Document and escalate promptly. **Objective tested: 10.**

Module 3 — Person-Centred Mealtimes and Independence

Lesson 7. Preferences, Culture and Dignity

Why this matters in professional caregiving

Food is connected to identity, culture, faith, memory, family and pleasure. A meal may be nutritious in theory but unacceptable, unfamiliar or undignified in practice. Respecting preferences can improve participation, while imposing foods or humiliating assistance can reduce intake and trust.

Learning objectives

The learner should be able to explore food preferences; respect cultural and religious practices; protect dignity; and avoid assumptions about what a Nigerian client eats.

Core instructional content

Ask about preferred foods, flavours, temperatures, portions, timing, utensils, seating, company and religious or personal practices. Consider sensory needs such as smell, colour, texture and temperature. Food refusal does not automatically mean stubbornness, laziness or lack of appetite.

Family involvement can be helpful, especially in learning history and preferences. However, a family member's preference is not automatically the client's current preference. Follow the care plan, consent and authorised restrictions. Do not shame someone for eating slowly, needing assistance or choosing familiar food.

Protect dignity by offering privacy where preferred, keeping clothing and surfaces clean, explaining assistance, avoiding infantilising language and not discussing how much the person ate in front of visitors. Support the person to make choices within safe options.

Applying This in Practice

Examples may include local staples, soups, fruits, legumes, fish, eggs or other foods familiar to the client, but no single menu should be assumed. Consider cost, availability, religious observance and household food practices. Ask rather than stereotype.

Case scenario

A client refuses a meal prepared by a family member and asks for a familiar alternative that is allowed by the care plan.

Correct professional response: Respect the preference, offer the authorised alternative if available, explore possible reasons and document repeated refusal or reduced intake. Do not shame or force.

Common risks or mistakes

Risks include cultural stereotyping, forcing family food, ignoring religious practice, humiliating a slow eater, dismissing preferences and confusing preference with clinical restriction.

Key practice points

1. Food is part of identity and dignity.
2. Ask rather than assume.
3. Family knowledge helps but does not replace the person's voice.
4. Refusal should be explored, not punished.

5. Follow authorised restrictions and plans.

Lesson assessment

1. **MCQ:** Person-centred mealtime support begins with: A) the caregiver's favourite menu; B) the person's preferences, plan and needs; C) family convenience only; D) a standard diet. **Answer: B. Objective tested: 2.**
2. **Scenario:** A client refuses an unfamiliar food. **Answer:** Explore, offer a suitable authorised alternative and document a meaningful pattern. **Objective tested: 2–4.**
3. **True/False:** All Nigerian clients share the same food preferences. **Answer: False. Objective tested: 2.**
4. **What should the caregiver NOT do?** Shame the client for eating slowly. **Answer:** Preserve dignity and allow safe time. **Objective tested: 2.**
5. **Application:** Name four preference areas to ask about. **Answer:** Taste, temperature, texture, portion, timing, utensils, company or religious practice. **Objective tested: 2.**
6. **Documentation:** Record current preference and any effect on intake. **Answer:** Correct. **Objective tested: 9.**

Lesson 8. Appetite and Food Refusal

Why this matters in professional caregiving

Reduced appetite can reflect illness, pain, nausea, constipation, dental problems, swallowing difficulty, medication effects, depression, emotional distress, fatigue, unfamiliar food, smell, temperature, environment, loneliness or difficulty feeding oneself. A caregiver who labels refusal as non-cooperation may miss an important need.

Learning objectives

The learner should be able to explore possible contributors to food refusal; use respectful alternatives; recognise sudden appetite loss; and escalate repeated or meaningful change.

Core instructional content

Begin by observing the context: what was offered, time, smell, temperature, texture, seating, people present, pain, nausea, toileting, fatigue, mood and ability to use utensils. Ask one simple question and allow time. Offer a suitable alternative only if it is authorised. A smaller portion or later attempt may help, but do not assume it is appropriate for every person.

Never force, threaten, bribe inappropriately, hide food, distract so the person cannot refuse or feed while the person is too drowsy. If the client refuses, document what was offered and

the response. Repeated refusal, sudden appetite loss, inability to eat, weight loss or associated symptoms should be reported.

Eating very slowly may reflect fatigue, pain, weakness, distraction, poor dentition or swallowing difficulty. Allow adequate time while observing safety. Do not rush or leave the person unattended if the plan requires supervision.

Applying This in Practice

In home care, relatives may say the client must finish. Explain that safe support includes listening and reporting. In facilities, compare patterns across shifts and avoid recording only “refused.”

Case scenario

A client suddenly refuses a favourite meal and appears nauseated. A caregiver considers forcing a few bites.

Correct professional response: Stop pressure, ask about discomfort, offer no unsafe improvisation, document the sudden change and escalate for appropriate assessment.

Common risks or mistakes

Risks include force, bribery, dismissing sudden change, ignoring dental or swallowing problems, rushing slow eating and failing to record what happened before refusal.

Key practice points

1. Food refusal has many possible contributors.
2. Sudden appetite loss deserves attention.
3. Never force or shame.
4. Allow time and adapt within the plan.
5. Document and report meaningful patterns.

Lesson assessment

1. **Scenario:** A client refuses a favourite meal suddenly. **Answer:** Explore discomfort, document the change and escalate. **Objective tested: 5.**
2. **MCQ:** Food refusal may be linked to: A) pain, nausea, dental or swallowing problems; B) only stubbornness; C) laziness; D) bad character. **Answer: A. Objective tested: 5.**
3. **True/False:** Force is acceptable if the meal is nutritious. **Answer: False. Objective tested: 2 and 5.**

4. **What should the caregiver NOT do?** Feed a drowsy person quickly. **Answer:** Stop and follow safety/escalation instructions. **Objective tested: 6 and 8.**
5. **Application:** Name four observations around refusal. **Answer:** Time, food, smell, temperature, pain, nausea, environment, fatigue, people present or ability to self-feed. **Objective tested: 5.**
6. **Scenario:** Eating becomes much slower over a week. **Answer:** Document and report for review. **Objective tested: 5 and 9.**

Lesson 9. Environment, Communication and Independence

Why this matters in professional caregiving

Noise, lighting, temperature, clutter, seating, distractions, social environment and pace can influence intake. Clients with dementia, visual or hearing impairment, mobility limitations or anxiety may need small environmental adaptations to participate safely.

Learning objectives

The learner should be able to prepare a supportive environment; use clear communication; select authorised adaptive aids; and support independence without substituting for the client.

Core instructional content

Prepare a stable seat and appropriate table height. Reduce unnecessary noise and competing conversations. Ensure adequate lighting without glare, place food and utensils where the person can see and reach them, and remove clutter. Introduce the meal calmly and explain one step at a time.

Support, not substitute. Adaptive utensils, cups, plate guards, non-slip mats, contrasting plates or package-opening assistance may help if approved and appropriate. Hand-over-hand support should occur only where authorised, welcomed and safe. Do not automatically feed a person who can eat with time or prompts.

Consider privacy and social preference. Some people eat better with company; others need a quieter space. Do not discuss weight, incontinence, disability or appetite publicly. Allow adequate time, but remain attentive to fatigue, coughing, food remaining in the mouth or other signs of risk.

Applying This in Practice

In Nigerian homes, generators, television, visitors, heat and household activity may affect mealtimes. Adapt the environment without treating family life as a problem. In residential

care, individualise rather than placing everyone in the same noisy dining space.

Case scenario

A client wants to eat independently but needs more time. The caregiver is tempted to take over because another task is waiting.

Correct professional response: Prepare the environment, offer prompts or an authorised aid, allow safe time and report staffing or scheduling problems that repeatedly undermine independence.

Common risks or mistakes

Risks include rushing, automatic feeding, unstable seating, poor lighting, clutter, unnecessary touch, loud correction and using unapproved aids.

Key practice points

1. Environment affects participation and safety.
2. Prepare before the meal starts.
3. Support, not substitute.
4. Allow time and observe fatigue.
5. Protect privacy and dignity.

Lesson assessment

1. **MCQ:** A helpful adaptation may be: A) clutter; B) appropriate lighting and accessible utensils; C) rushing; D) several people giving instructions. **Answer: B. Objective tested: 8.**
2. **Scenario:** The client can self-feed with extra time. **Answer:** Support and allow time rather than taking over. **Objective tested: 2 and 8.**
3. **True/False:** Every client eats best in a busy communal setting. **Answer: False. Objective tested: 2.**
4. **What should the caregiver NOT do?** Use hand-over-hand feeding without authorisation or consent. **Answer:** Follow the plan and respect the person. **Objective tested: 8.**
5. **Application:** Name four environmental factors. **Answer:** Noise, lighting, temperature, seating, clutter, distractions, social setting, privacy or pace. **Objective tested: 8.**
6. **Scenario:** The client becomes tired halfway through eating. **Answer:** Pause safely, observe, offer appropriate support and document/report if new or repeated. **Objective**

tested: 5 and 9.

Module 4 — Malnutrition and Swallowing Awareness

Lesson 10. Malnutrition Risk

Why this matters in professional caregiving

Malnutrition risk can develop when a person eats less than needed, cannot access or prepare food, loses weight unintentionally or becomes too weak to eat independently. Caregivers may notice changes before a formal assessment occurs.

Learning objectives

The learner should be able to recognise possible warning signs; identify contributing barriers; document patterns; and seek appropriate assessment without diagnosing.

Core instructional content

Possible warning signs include unintentional weight loss, poor appetite, reduced food intake, weakness, fatigue, ill-fitting clothes, reduced muscle strength, difficulty shopping or preparing food, increased dependence and repeated missed meals. These are warning signs, not a diagnosis. A person may also be at risk despite appearing to have a larger body size.

Consider barriers: pain, nausea, constipation, dental problems, swallowing difficulty, depression, fatigue, poverty, isolation, inaccessible food, inability to open packages, cultural mismatch or a care schedule that misses meals. Support immediate access within role and report the pattern.

Validated screening tools may help authorised teams identify risk, but caregivers should use them only with local policy and training. Do not assign a score, diagnose malnutrition or start supplements independently.

Applying This in Practice

Use a food-and-fluid observation record and report changes through the care pathway. In home care, note shopping and preparation barriers. In residential care, share patterns across shifts and review repeated missed meals.

Case scenario

A client's clothes are loose, the client is weak and three meals have been partly refused. A caregiver says, "They are just getting older."

Correct professional response: Notice the pattern, document and report for appropriate assessment. Do not normalise or diagnose.

Common risks or mistakes

Risks include normalising weight loss, using body size as proof, prescribing supplements, completing a screening tool without training and recording only one missed meal.

Key practice points

1. Notice, document, report and seek assessment.
2. Weight loss and weakness are warning signs, not proof.
3. Look for access, health, emotional and swallowing barriers.
4. Do not prescribe supplements or diagnose.
5. Escalate repeated or worsening change.

Lesson assessment

1. **MCQ:** Possible malnutrition warning signs include: A) unintentional weight loss and weakness; B) only low body weight; C) a single disliked meal; D) good appetite. **Answer:** A. **Objective tested:** 4.
2. **Scenario:** Clothes are becoming loose and intake is reduced. **Answer:** Document and report for assessment. **Objective tested:** 4 and 9.
3. **True/False:** A caregiver may diagnose malnutrition from appearance. **Answer:** False. **Objective tested:** 4.
4. **What should the caregiver NOT do?** Start supplements without authorisation. **Answer:** Seek appropriate guidance. **Objective tested:** 3–4.
5. **Application:** Name four possible contributors. **Answer:** Pain, nausea, dental problems, swallowing difficulty, fatigue, access, isolation or depression. **Objective tested:** 4.
6. **Screening question:** Use validated tools only under local policy and training. **Answer:** Correct. **Objective tested:** 4.
7. **Documentation:** Record intake pattern, weight-related observation where authorised, weakness, barriers and escalation. **Answer:** Correct. **Objective tested:** 9.

Lesson 11. Swallowing and Aspiration Awareness

Why this matters in professional caregiving

Swallowing problems can create risk of choking, aspiration, dehydration and inadequate nutrition. Caregivers do not diagnose dysphagia, but they may observe warning signs and prevent unsafe continuation of a meal.

Learning objectives

The learner should be able to explain swallowing awareness; recognise possible warning signs; stop unsafe support; follow the care plan; and escalate.

Core instructional content

Possible warning signs include coughing during meals, repeated throat clearing, wet or gurgly voice after eating or drinking, prolonged chewing, food remaining in the mouth, apparent difficulty swallowing, repeated chest infections where known, unexplained weight loss and avoidance of certain textures. No single sign proves dysphagia, and some people show few visible signs.

If a person coughs repeatedly, becomes distressed, cannot clear food, develops a wet voice or appears unsafe, stop the support and follow the authorised pathway. Do not continue giving “small tests,” reach into the mouth blindly, change the texture, thicken a drink or offer a substitute without guidance.

Positioning, pace, alertness, texture and supervision should follow the plan. A person who is too drowsy to eat safely should not be fed simply to complete the schedule; seek guidance.

Applying This in Practice

In homes and facilities, share warning signs at handover. In a Nigerian context, family members may offer familiar foods or home remedies; acknowledge the concern but follow the authorised plan and escalate.

Case scenario

A client develops a wet voice and food remains in the mouth after a meal. A family member says to give water to “wash it down.”

Correct professional response: Stop unsafe support, keep the person safe, follow the authorised swallowing plan and report promptly. Do not give water or continue testing without instruction.

Common risks or mistakes

Risks include diagnosing, continuing despite coughing, giving water automatically, sweeping the mouth with fingers, changing textures and ignoring fatigue or drowsiness.

Key practice points

1. Recognise possible signs; do not diagnose.
2. Stop unsafe support.
3. Follow the care plan exactly.
4. Report swallowing concerns promptly.
5. Do not improvise drinks, textures or oral clearing.

Lesson assessment

1. **MCQ:** A possible swallowing concern is: A) wet/gurgly voice after drinking; B) a preferred taste; C) clean hands; D) a normal swallow. **Answer: A. Objective tested: 6.**
2. **Scenario:** Food remains in the mouth. **Answer:** Stop unsafe support, follow the plan and escalate. **Objective tested: 6–7.**
3. **True/False:** One sign confirms dysphagia. **Answer: False. Objective tested: 6.**
4. **What should the caregiver NOT do?** Give water automatically to clear food. **Answer:** It may be unsafe; seek authorised guidance. **Objective tested: 6–7.**
5. **Application:** Name four warning signs. **Answer:** Coughing, throat clearing, wet voice, prolonged chewing, pocketing food, difficulty swallowing, weight loss or texture avoidance. **Objective tested: 6.**
6. **Scenario:** The client is very drowsy at mealtime. **Answer:** Do not feed automatically; follow the plan and seek guidance. **Objective tested: 7–8.**
7. **Documentation:** Record sign, food/fluid, timing, action, outcome and person informed. **Answer:** Correct. **Objective tested: 9–10.**

Lesson 12. Texture-Modified Diets and Safe Escalation

Why this matters in professional caregiving

Some clients have authorised instructions for pureed, soft, minced, modified-texture foods, thickened fluids or other specialised diets. A small change in texture can alter safety. Caregivers must follow the plan exactly and must not improvise when products are unavailable.

Learning objectives

The learner should be able to identify texture-modified plans; follow them accurately; recognise when required food or fluid is unavailable; and escalate without changing consistency independently.

Core instructional content

A texture plan may specify food consistency, fluid thickness, utensils, positioning, pace, supervision and actions if coughing occurs. Read the current plan before serving. Confirm that staff understand the same instructions and that the available food or drink matches the plan.

Do not thicken drinks, blend foods, dilute fluids, add flour, use household substitutes, decide that a food is “soft enough,” change portion texture or offer an unapproved alternative. “STOP → DO NOT IMPROVISE → FOLLOW THE ESCALATION PATHWAY.”

If a client refuses a modified texture, do not provide a regular texture secretly. Explore preference respectfully and report the refusal or barrier. A responsible professional may need to review the plan, equipment, menu or training.

Applying This in Practice

In home care, label and separate texture products clearly. In residential care, include the plan in handover and check kitchen communication. During outages or supply shortages, escalate early rather than improvising.

Case scenario

A client’s thickened fluid product is unavailable and the caregiver is told to add flour.

Correct professional response: Do not improvise. Stop the uncertain support, contact the authorised person and document the supply problem.

Common risks or mistakes

Risks include household thickener, guessing consistency, unapproved substitutions, cross-contamination, unclear handover and serving the wrong meal.

Key practice points

1. Follow texture and fluid plans exactly.
2. Do not change consistency independently.
3. Stop when the required item is unavailable.
4. Escalate supply and plan problems.
5. Document refusal, mismatch or unsafe signs.

Lesson assessment

1. **MCQ:** A texture-modified plan must be: A) interpreted casually; B) followed exactly; C) replaced by family advice; D) changed when inconvenient. **Answer: B. Objective**

tested: 7.

2. **Scenario:** Required thickener is unavailable. **Answer:** Stop, do not improvise and follow the escalation pathway. **Objective tested: 7 and 10.**
3. **True/False:** A caregiver may blend a meal without authorisation. **Answer: False.**
Objective tested: 7.
4. **What should the caregiver NOT do?** Use flour as a substitute thickener. **Answer:** It may be unsafe and is unauthorised. **Objective tested: 7.**
5. **Application:** Name three plan elements to check. **Answer:** Food texture, fluid thickness, position, utensils, pace, supervision or response to coughing. **Objective tested: 7–8.**
6. **Scenario:** A client refuses modified food. **Answer:** Respect, document and seek review; do not secretly serve regular food. **Objective tested: 7 and 9.**
7. **Documentation:** Record mismatch, action, person informed and interim safety decision. **Answer:** Correct. **Objective tested: 9–10.**

Module 5 — Choking, Positioning and Food Safety

Lesson 13. Safe Positioning and Mealtime Attention

Why this matters in professional caregiving

Position and attention influence safety, comfort, dignity and the caregiver's ability to notice problems. Basic caregiver-level positioning should remain within the care plan. Complex swallowing manoeuvres or specialist positioning require qualified instruction.

Learning objectives

The learner should be able to prepare stable seating; support upright positioning where appropriate; maintain attention; and avoid unsafe feeding when the person is drowsy or poorly positioned.

Core instructional content

Use stable seating, appropriate table height and the positioning specified in the plan. Where appropriate, the person should be upright and supported rather than eating while lying down. Keep the environment calm enough to observe. Remain attentive to coughing, fatigue, wet voice, food remaining in the mouth and distress.

Do not force the head, neck or jaw into a position. Do not teach or perform complex swallowing manoeuvres unless separately trained and authorised. If the person slides,

becomes drowsy, cannot maintain a safe position or shows swallowing concern, pause and seek guidance.

Applying This in Practice

Check chairs, cushions, foot support and table height. In homes, consider uneven floors, low furniture and distractions. In facilities, avoid feeding in corridors or while the person is being moved.

Case scenario

A client begins sliding down in the chair and becomes sleepy while eating.

Correct professional response: Pause feeding, maintain safety, follow the plan and seek assistance. Do not continue to finish the meal.

Common risks or mistakes

Risks include feeding lying down, unstable seating, leaving a person unattended, rushing, forceful positioning and complex manoeuvres outside training.

Key practice points

1. Prepare stable seating and an appropriate table.
2. Use upright positioning where appropriate and authorised.
3. Remain attentive throughout support.
4. Pause if alertness or positioning becomes unsafe.
5. Do not perform specialist manoeuvres outside training.

Lesson assessment

1. **MCQ:** A basic safe-mealtime principle is: A) stable seating and appropriate positioning; B) feeding while lying down; C) leaving the person alone; D) rushing. **Answer: A.**
Objective tested: 8.
2. **Scenario:** The person becomes drowsy. **Answer:** Pause support and seek guidance.
Objective tested: 8.
3. **True/False:** A caregiver should force the head into position. **Answer: False. Objective tested: 8.**
4. **What should the caregiver NOT do?** Continue feeding to complete the target despite unsafe posture. **Answer:** Safety takes priority. **Objective tested: 8–10.**

5. **Application:** Name four things to check. **Answer:** Seating, posture, table height, foot support, alertness, distractions or attention. **Objective tested: 8.**
6. **Documentation:** Record the unsafe change, pause, assistance and escalation. **Answer:** Correct. **Objective tested: 9–10.**

Lesson 14. Choking Awareness and Emergency Response

Why this matters in professional caregiving

Choking is an emergency. Caregivers should understand the difference between effective coughing and severe airway obstruction, activate help promptly and follow the recognised first-aid procedure for which they are trained. Online CPD cannot replace hands-on first-aid practice.

Learning objectives

The learner should be able to distinguish coughing from severe obstruction at an awareness level; activate emergency help; follow trained procedure; and document and escalate afterward.

Core instructional content

A person who is coughing effectively may still be moving air. Encourage them to cough and monitor closely; do not automatically perform thrusts on an effective cough. Signs of severe airway obstruction may include inability to speak, breathe or cough effectively, silent attempts to cough, increasing distress or collapse.

For a severe obstruction, seek help immediately, activate the local emergency response and follow the current first-aid procedure taught by a recognised authority and applicable to the person. The American Red Cross adult/child guidance includes back blows and abdominal thrusts for appropriate cases, but exact manoeuvres, adaptations and contraindications require current hands-on training ⁶. Do not invent or improvise a protocol from this CPD.

Do not leave a person in a life-threatening situation while seeking routine supervisor approval. Once the emergency is managed, obtain medical review according to local procedure, document the event and report it. A person who recovers may still require assessment.

Applying This in Practice

NIC should link or require a recognised current first-aid course for roles expected to respond physically to choking. Local emergency numbers, facility procedures and

adaptations for pregnancy, disability or age must be taught through the appropriate practical programme.

Case scenario

A client suddenly cannot speak or cough and appears distressed. The caregiver is not sure whether it is choking.

Correct professional response: Treat as a possible life-threatening obstruction, call for emergency help immediately and follow the trained first-aid procedure. Do not wait for routine approval or search the internet during the emergency.

Common risks or mistakes

Risks include confusing effective cough with complete obstruction, blind finger sweeps, delaying help, using an untrained technique, leaving the person alone and assuming recovery means no review is needed.

Key practice points

1. Choking is an emergency.
2. Activate help promptly.
3. Follow trained, recognised first-aid procedure.
4. Do not invent manoeuvres or delay for routine approval.
5. Obtain follow-up and document the event.

Lesson assessment

1. **MCQ:** If a person is coughing effectively, the caregiver should generally: A) encourage coughing and monitor; B) immediately force fluids; C) perform random thrusts; D) leave. **Answer: A. Objective tested: 6 and 8.**
2. **Scenario:** The person cannot speak or cough effectively. **Answer:** Activate emergency response and follow trained procedure. **Objective tested: 10.**
3. **True/False:** Online text alone proves hands-on choking competence. **Answer: False. Objective tested: 8.**
4. **What should the caregiver NOT do?** Perform a blind finger sweep. **Answer:** It can cause harm; follow trained procedure. **Objective tested: 8.**
5. **Application:** Name three emergency actions. **Answer:** Call for help, activate emergency response, follow trained procedure, stay with the person and arrange review. **Objective tested: 10.**

6. **Scenario:** The person appears recovered after choking. **Answer:** Follow medical review and incident-reporting procedures. **Objective tested: 9–10.**
7. **Scope question:** Where should physical manoeuvres be learned? **Answer:** Recognised, current hands-on first-aid training. **Objective tested: 8.**

Lesson 15. Food Safety in Care Settings

Why this matters in professional caregiving

Clients may be more vulnerable to consequences of foodborne illness. Caregivers often handle food in homes, kitchens, care facilities and during transport. Simple controls can reduce contamination and spoilage.

Learning objectives

The learner should be able to apply caregiver-level food-safety principles; identify expiry, storage and contamination risks; and report unsafe food.

Core instructional content

WHO's Five Keys to Safer Food are: **keep clean; separate raw and cooked; cook thoroughly; keep food at safe temperatures; and use safe water and raw materials** 5 .

Caregivers should wash hands appropriately, keep preparation surfaces and utensils clean, separate raw and ready-to-eat food, check expiry and storage instructions, reheat safely according to policy, protect food from contamination and handle leftovers through the approved process.

Food should not be served if it is expired, visibly spoiled, contaminated, improperly stored or of uncertain safety. Do not taste questionable food to test it. If an electricity outage affects refrigeration, follow the facility or food-safety procedure and seek guidance rather than guessing that food is safe.

Personal hygiene includes clean hands, appropriate clothing and not preparing food while experiencing symptoms or contamination risks covered by policy. Do not prepare food near chemicals, raw waste or dirty equipment.

Applying This in Practice

In Nigerian homes and facilities, consider unreliable electricity, heat, open markets, transport time, water supply and shared kitchens. The principle is not to condemn local practice but to use safe storage, clean water, separation and timely escalation.

Case scenario

Power has been out for many hours and refrigerated food is warm. A caregiver is told to serve it because “it smells fine.”

Correct professional response: Do not serve uncertain food. Isolate it according to policy, report the storage failure and seek an authorised food-safety decision.

Common risks or mistakes

Risks include serving expired food, tasting questionable food, mixing raw and cooked foods, leaving food warm too long, using dirty water and ignoring refrigeration failure.

Key practice points

1. Keep clean.
2. Separate raw and cooked.
3. Cook thoroughly and keep food at safe temperatures.
4. Use safe water and raw materials.
5. Escalate uncertainty rather than guessing.

Lesson assessment

1. **MCQ:** Which is one WHO Five Key? A) keep raw and cooked together; B) keep clean; C) taste spoiled food; D) ignore temperature. **Answer: B. Objective tested: 8.**
2. **Scenario:** Food is expired. **Answer:** Do not serve; isolate/report according to procedure. **Objective tested: 8 and 10.**
3. **True/False:** Smell alone proves food is safe. **Answer: False. Objective tested: 8.**
4. **What should the caregiver NOT do?** Mix raw chicken preparation tools with ready-to-eat food. **Answer:** Prevent cross-contamination. **Objective tested: 8.**
5. **Application:** Name three actions after a power outage affects refrigeration. **Answer:** Stop serving uncertain food, isolate/label, report and follow food-safety guidance. **Objective tested: 8–10.**
6. **Documentation:** Record the food, time, storage concern, action and person informed. **Answer:** Correct. **Objective tested: 9.**
7. **Scenario:** Water safety is uncertain. **Answer:** Do not use it for food or drinks until authorised safe water is available. **Objective tested: 8.**

Module 6 — Allergies, Restrictions, Documentation and Escalation

Lesson 16. Allergies, Intolerances and Dietary Restrictions

Why this matters in professional caregiving

A known food allergy can cause a rapid and serious reaction. An intolerance may cause unpleasant symptoms but is not the same as an immune-mediated allergy. Caregivers must not diagnose either condition, but they must check authorised information, prevent avoidable exposure and escalate suspected reactions.

Learning objectives

The learner should be able to distinguish allergy awareness from diagnosis; check care-plan information; prevent cross-contact; manage unapproved alternatives; and escalate reactions.

Core instructional content

Before serving, check the current care plan, allergy record, dietary restriction and food label or kitchen information. Confirm ingredients when required. Do not assume that a familiar food is safe or that removing the visible ingredient removes cross-contact risk.

If a family member brings an alternative food that has not been approved, do not serve it until the authorised information is checked. If the client has a known allergy and exposure is suspected, follow the emergency plan and activate urgent help for signs such as breathing difficulty, swelling, widespread rash, collapse or severe vomiting. Do not diagnose or delay while debating whether it is “only intolerance.”

Restrictions may arise from allergy, intolerance, faith, ethics, clinical treatment, texture, fluid plan or personal preference. Follow the documented instruction and escalate unclear or conflicting information.

Applying This in Practice

In homes, shared cooking surfaces and family food can create cross-contact. In facilities, use clear labels, separate utensils and reliable handover. Do not rely on memory.

Case scenario

A client has a documented peanut allergy. A family member brings a cake with no ingredient label and says it contains no nuts.

Correct professional response: Do not serve it until authorised ingredients and safety are confirmed. Document the issue and inform the responsible person.

Common risks or mistakes

Risks include relying on verbal reassurance, serving unlabelled food, cross-contact, diagnosing intolerance, ignoring religious restrictions and failing to escalate reaction signs.

Key practice points

1. Check the current authorised information before serving.
2. Prevent cross-contact.
3. Do not diagnose allergy or intolerance.
4. Do not serve unapproved alternatives.
5. Treat suspected serious reactions as urgent.

Lesson assessment

1. **MCQ:** An allergy differs from intolerance because: A) they are identical; B) allergy may involve an immune reaction and serious emergency risk; C) intolerance is always fatal; D) caregivers can diagnose both. **Answer: B. Objective tested: 16.**
2. **Scenario:** An unlabelled alternative is offered to a client with a known allergy. **Answer:** Do not serve; check authorised information and escalate uncertainty. **Objective tested: 16.**
3. **True/False:** Removing a visible ingredient always removes cross-contact risk. **Answer: False. Objective tested: 16.**
4. **What should the caregiver NOT do?** Wait to see whether breathing difficulty improves. **Answer:** Activate urgent emergency response according to the plan. **Objective tested: 10.**
5. **Application:** Name three checks before serving. **Answer:** Care plan, allergy record, ingredients/label, texture, fluid and restriction information. **Objective tested: 16.**
6. **Scenario:** The plan and family instruction conflict. **Answer:** Follow the authorised plan and seek clarification; do not guess. **Objective tested: 7 and 16.**
7. **Documentation:** Record food, suspected exposure, signs, actions and notifications. **Answer:** Correct. **Objective tested: 9–10.**

Lesson 17. Documentation and Handover

Why this matters in professional caregiving

A useful record supports continuity, identifies patterns and helps professionals assess risk.

“Ate poorly” is less useful than a clear description of what was offered, taken, refused, observed and reported.

Learning objectives

The learner should be able to document food and fluid observations objectively; record swallowing, allergy and safety concerns; provide handover; and maintain confidentiality.

Core instructional content

Record date, time, meal or drink, amount where required by policy, assistance provided, refusal, coughing or other signs, fatigue, positioning, environmental factors, outcome, person informed and follow-up. Use exact words when relevant. Separate facts from interpretation: “Client coughed four times after two sips” is preferable to “client cannot swallow.”

Document what helped as well as what went wrong. Record patterns over time, not just isolated events. If a serious event occurs, make direct escalation in addition to completing the incident record. Use approved systems and protect records from unauthorised access.

Handover should state current risk, what happened, what support is authorised, what was effective, what should be avoided and who is following up. Do not use family gossip, public messaging or personal notes as the permanent record.

Applying This in Practice

Use NIC or employer-approved Food & Fluid Observation Record and Mealtime Risk Checklist. In home care, communicate necessary information without exposing the client’s private details to unrelated family members.

Case scenario

A record says, “Client did not cooperate with lunch.” The next caregiver does not know that the client coughed and became tired.

Correct professional response: Correct or add an objective record through the approved process and communicate the swallowing and fatigue observations promptly.

Common risks or mistakes

Risks include vague labels, delayed reporting, backdating, deleting entries, documenting only amounts, ignoring signs and sharing records through personal messaging.

Key practice points

1. Record facts, context, response and outcome.
2. Include signs and barriers, not only quantities.
3. Escalate urgent risk directly.

4. Handover what to do and what to avoid.
5. Use secure approved systems.

Lesson assessment

1. **MCQ:** Which is objective? A) “Client was difficult” ; B) “Client refused half the meal and coughed twice” ; C) “Client is malnourished” ; D) “Family is careless.” **Answer: B. Objective tested: 9.**
2. **Scenario:** Coughing is omitted from the record. **Answer:** Add objective information through the approved process and escalate. **Objective tested: 6 and 9.**
3. **True/False:** A completed form always replaces urgent verbal escalation. **Answer: False. Objective tested: 10.**
4. **What should the caregiver NOT do?** Use a personal WhatsApp group for handover. **Answer:** Use approved confidential systems. **Objective tested: 9.**
5. **Application:** Name five record elements. **Answer:** Time, food/fluid, amount, signs, support, response, outcome, person informed or follow-up. **Objective tested: 9.**
6. **Scenario:** A family asks for the complete record. **Answer:** Follow authorisation and privacy policy; do not disclose automatically. **Objective tested: 9.**
7. **Handover:** State current risk, effective support and next action. **Answer:** Correct. **Objective tested: 9–10.**

Lesson 18. Integrated Mealtime Risk Judgement

Why this matters in professional caregiving

Mealtime situations combine nutrition, hydration, choice, swallowing, allergy, food safety, dignity, family pressure and documentation. The caregiver needs a repeatable method for deciding what to do first and when to stop.

Learning objectives

The learner should be able to apply KNOW–CARE; prioritise safety; preserve independence; avoid unsafe improvisation; document and escalate.

Core instructional content

Start by knowing the person and checking the plan. Prepare safely. Support choice and independence. Observe intake and signs. Recognise risk without diagnosing. Respond only within scope. Document and report.

When a concern appears, ask: What is the concern? What should I observe? What can I safely do? What must I not do? What should be documented? Who should be informed? Is it urgent? Is it within my role?

Stop unsafe support when there is possible choking, repeated coughing, wet voice, unsafe alertness, allergic reaction, wrong texture, contaminated food, sudden deterioration or a serious change in intake. Protect the person, activate the relevant emergency or escalation pathway and do not wait for routine approval where there is life-threatening danger.

Applying This in Practice

Use the framework in homes, facilities, hospitals and community services. It should become shared language for supervision, handover and incident review.

Case scenario

A client with a modified-fluid plan is unusually drowsy, has taken little fluid, coughs after a drink and a relative offers an unlabelled snack. The caregiver is asked to continue because the family is busy.

Correct professional response: Pause unsafe support; check the authorised plan; do not serve the unlabelled snack or continue testing; protect positioning and safety; escalate the swallowing, alertness and low-intake concerns; document facts and notifications; and maintain dignity and confidentiality.

Common risks or mistakes

Risks include prioritising meal completion over safety, serving unknown food, changing texture, forcing intake, failing to report acute change and documenting only “poor appetite.”

Key practice points

1. Use a structured pause.
2. Safety and dignity come before meal completion.
3. Follow authorised plans exactly.
4. Never improvise texture, fluid or allergy decisions.
5. Document and escalate meaningful risk.

Lesson assessment

1. **Integrated scenario:** Drowsiness, coughing and low intake occur together. **Answer:** Stop unsafe support, protect the person and escalate promptly. **Objective tested: 6–10.**

2. **MCQ:** The first step in KNOW–CARE is: A) prescribe; B) know the person and plan; C) force; D) document a diagnosis. **Answer: B. Objective tested: 10.**
3. **True/False:** Completing the meal is more important than safe swallowing. **Answer: False. Objective tested: 6–8.**
4. **What should the caregiver NOT do?** Serve an unlabelled snack to a client with an allergy. **Answer:** Verify authorised safety first. **Objective tested: 16.**
5. **Application:** List the escalation triggers in the scenario. **Answer:** Drowsiness, coughing, modified-fluid plan, low intake and unlabelled food. **Objective tested: 6–10.**
6. **Documentation:** Record plan, signs, food/fluid, support stopped, people informed and next action. **Answer:** Correct. **Objective tested: 9–10.**
7. **Scope:** What must not be done? Diagnose, change texture, prescribe fluids, choose an allergy substitute or delay urgent help. **Answer:** Correct. **Objective tested: 1, 3, 7 and 10.**

13. Lesson assessments

Lesson assessments are embedded after each lesson. NIC should provide immediate feedback explaining the professional reasoning, not only the correct answer. Equivalent items may be randomised while keeping the same learning objective and safety standard.

14. Module assessments

Each module assessment should contain 5–8 questions, with an 80% recommended threshold and one retry after review.

Module 1 assessment

1. **Scenario:** Intake falls for three days. **Answer:** Observe, document and report; do not prescribe. **LO:** 1, 4, 9.
2. **MCQ:** Protein broadly supports tissue maintenance and repair. **Answer:** Correct at awareness level. **LO:** 1.
3. **Scenario:** Family requests an unapproved therapeutic diet change. **Answer:** Do not change; seek authorised guidance. **LO:** 1, 10.
4. **True/False:** General nutrition awareness is the same as clinical dietary management. **Answer:** False. **LO:** 1, 7.
5. **What should not happen?** Independent supplement prescription. **LO:** 1, 3.
6. **Application:** Document barriers such as pain, access, dental problems or swallowing

concerns. **Answer:** Correct. **LO:** 4–5.

Module 2 assessment

1. **Scenario:** Client drinks little because the cup is difficult to hold. **Answer:** Adapt access within role and record intake. **LO:** 3.
2. **MCQ:** Dry mouth is a possible sign, not proof, of dehydration. **Answer:** Correct. **LO:** 3.
3. **Scenario:** Repeated vomiting and weakness. **Answer:** Escalate promptly. **LO:** 3, 10.
4. **True/False:** Caregivers may prescribe individual fluid volumes. **Answer:** False. **LO:** 3.
5. **What should not happen?** Household thickener for coughing fluids. **LO:** 6–7.
6. **Application:** Use Observe → Record → Report. **Answer:** Correct. **LO:** 3, 9.

Module 3 assessment

1. **Scenario:** Client refuses an unfamiliar meal. **Answer:** Explore, offer authorised alternative and document pattern. **LO:** 2, 5.
2. **MCQ:** A supportive environment reduces avoidable noise and clutter. **Answer:** Correct. **LO:** 8.
3. **Scenario:** Client can self-feed with extra time. **Answer:** Support, not substitute. **LO:** 2, 8.
4. **True/False:** Family preference automatically overrides the client's current preference. **Answer:** False. **LO:** 2.
5. **What should not happen?** Force or shame. **LO:** 2, 5.
6. **Application:** Record successful environmental adaptations. **Answer:** Correct. **LO:** 9.

Module 4 assessment

1. **Scenario:** Loose clothes, weakness and repeated missed meals. **Answer:** Document and report for assessment. **LO:** 4.
2. **MCQ:** Wet voice after drinking may signal swallowing concern. **Answer:** Correct. **LO:** 6.
3. **Scenario:** Required thickener unavailable. **Answer:** Stop, do not improvise, escalate. **LO:** 7, 10.
4. **True/False:** One cough diagnoses dysphagia. **Answer:** False. **LO:** 6.
5. **What should not happen?** Giving water to clear pocketed food without instruction. **LO:** 6–7.
6. **Application:** Record warning signs, response and person informed. **Answer:** Correct. **LO:** 9–10.

7. **Scenario:** Client refuses modified food. **Answer:** Respect, document and seek review. **LO:** 7.

Module 5 assessment

1. **Scenario:** Client slides down and becomes drowsy. **Answer:** Pause feeding and seek help. **LO:** 8.
2. **MCQ:** Effective coughing is different from severe obstruction. **Answer:** Correct. **LO:** 6, 8.
3. **Scenario:** Person cannot speak or cough. **Answer:** Activate emergency response and follow trained procedure. **LO:** 10.
4. **True/False:** Online CPD replaces hands-on first-aid training. **Answer:** False. **LO:** 8.
5. **What should not happen?** Serve food affected by uncertain refrigeration. **LO:** 8.
6. **Application:** Apply WHO Five Keys. **Answer:** Keep clean; separate raw/cooked; cook thoroughly; safe temperatures; safe water/raw materials. **LO:** 8.
7. **Documentation:** Record food-safety failure and escalation. **Answer:** Correct. **LO:** 9–10.

Module 6 assessment

1. **Scenario:** Unlabelled cake offered to client with known allergy. **Answer:** Do not serve until verified. **LO:** 7, 10.
2. **MCQ:** Allergy and intolerance should not be diagnosed by caregivers. **Answer:** Correct. **LO:** 7.
3. **Scenario:** Record says “uncooperative” but client coughed. **Answer:** Add objective facts and escalate. **LO:** 9–10.
4. **True/False:** A completed form always replaces urgent verbal escalation. **Answer:** False. **LO:** 10.
5. **What should not happen?** Sharing client food records in a personal group. **LO:** 9.
6. **Integrated scenario:** Low intake plus coughing plus wrong texture. **Answer:** Stop unsafe support, follow plan, escalate and document. **LO:** 4, 6, 7, 9–10.
7. **Application:** Use KNOW–CARE in order. **Answer:** Know, Notice, Organise, Welcome, Carefully observe, Act within scope, Record, Escalate. **LO:** 10.

15. Final assessment blueprint

The final assessment contains **35 questions**. The recommended pass mark is **80% (28/35)**. At least **25 questions are scenario/application based**, exceeding the requested minimum of 23.

Content area	Questions	Number	Application target
Nutrition and hydration foundations	1–5	5	3
Person-centred mealtime support	6–10	5	4
Hydration and malnutrition risk	11–16	6	5
Swallowing awareness and choking	17–22	6	5
Food safety, allergies and restrictions	23–27	5	4
Dementia, disability and independence at mealtimes	28–31	4	3
Documentation, escalation and integrated judgement	32–35	4	4
Total		35	28

16. Final assessment questions

1. **MCQ:** The caregiver's nutrition role is to: A) prescribe diets; B) support the authorised plan, observe intake and report concerns; C) diagnose malnutrition; D) calculate every client's calorie need. **Answer: B. LO: 1.**
2. **Scenario:** A client eats much less than usual for three days. What is the best next action? A) Force food; B) prescribe supplements; C) document, observe possible contributors and report; D) ignore it. **Answer: C. LO: 1, 4, 9.**
3. **MCQ:** Protein is broadly associated with: A) tissue maintenance and repair; B) replacing fluids; C) curing every illness; D) automatic supplementation. **Answer: A. LO: 1.**
4. **Scenario:** A relative asks the caregiver to remove salt because of a recent blood-pressure reading, but no authorised instruction exists. **Answer:** Do not change the diet; seek guidance. **LO: 1, 3.**
5. **True/False:** A balanced meal means one fixed menu for all clients. **Answer: False. LO: 1–2.**

6. **Scenario:** A client refuses an unfamiliar meal but requests an authorised familiar alternative. **Answer:** Offer the alternative if available, respect the preference and document repeated refusal. **LO:** 2, 5.
7. **MCQ:** A person-centred mealtime includes: A) choice, dignity, culture and independence; B) speed only; C) family control only; D) forced completion. **Answer: A. LO:** 2.
8. **Scenario:** A family member insists the client must finish the plate. **Answer:** Explain that safe support respects refusal, explore the reason and report meaningful reduced intake. **LO:** 2, 5.
9. **What should the caregiver NOT do?** Shame a client for slow eating or needing assistance. **Answer:** Protect dignity and allow safe time. **LO:** 2, 8.
10. **Scenario:** The client can self-feed with an adaptive cup and extra time. **Answer:** Support, not substitute. **LO:** 2, 8.
11. **MCQ:** A dry mouth is: A) definitive proof of dehydration; B) a possible concern requiring context; C) irrelevant; D) a reason to force fluids. **Answer: B. LO:** 3.
12. **Scenario:** A client drinks little because the cup is difficult to hold. **Answer:** Provide an authorised accessible cup or aid, offer fluids appropriately and record intake. **LO:** 3.
13. **Scenario:** A client has weak appearance, dark urine where observable and very low intake. **Answer:** Document and escalate; do not prescribe a fluid volume. **LO:** 3, 10.
14. **True/False:** The caregiver may independently set a fluid target. **Answer: False. LO:** 3.
15. **Scenario:** Clothes are loose, the client is weak and several meals have been partly refused. **Answer:** Notice, document, report and seek appropriate assessment. **LO:** 4.
16. **What should the caregiver NOT do?** Start nutritional supplements without authorisation. **Answer:** Clinical nutrition decisions require appropriate professional guidance. **LO:** 1, 4.
17. **MCQ:** A possible swallowing warning sign is: A) wet or gurgly voice after drinking; B) a preferred taste; C) clean hands; D) a normal appetite. **Answer: A. LO:** 6.
18. **Scenario:** A client coughs repeatedly while drinking. **Answer:** Stop unsafe support, follow the plan and report promptly. **LO:** 6–7.
19. **Scenario:** Food remains in the client's mouth and a relative says to give water to clear it. **Answer:** Do not give water automatically; stop and seek authorised guidance. **LO:** 6–7.
20. **True/False:** One cough confirms dysphagia. **Answer: False. LO:** 6.
21. **Scenario:** Thickened-fluid product is unavailable. **Answer:** Stop, do not improvise and follow the escalation pathway. **LO:** 7, 10.

22. **What should the caregiver NOT do?** Blend, dilute or thicken food or fluids independently. **Answer:** Follow the authorised plan exactly. **LO:** 7.
23. **MCQ:** Which is a WHO Five Key to Safer Food? A) keep raw and cooked foods together; B) keep clean; C) taste questionable food; D) ignore temperature. **Answer:** B. **LO:** 8.
24. **Scenario:** Refrigerated food is warm after a prolonged power outage. **Answer:** Do not serve uncertain food; isolate/report and seek an authorised food-safety decision. **LO:** 8, 10.
25. **Scenario:** A client has a known peanut allergy and an unlabelled cake is offered. **Answer:** Do not serve until ingredients and safety are authorised. **LO:** 7, 10.
26. **True/False:** Removing a visible allergen always removes cross-contact risk. **Answer:** False. **LO:** 7.
27. **Scenario:** The client develops breathing difficulty and swelling after suspected food exposure. **Answer:** Activate urgent emergency response according to the plan; do not wait or diagnose. **LO:** 7, 10.
28. **Scenario:** A person with visual impairment cannot locate the plate. **Answer:** Improve contrast/positioning and provide authorised prompts while preserving independence. **LO:** 8.
29. **Scenario:** A client with dementia becomes confused in a noisy dining room. **Answer:** Reduce distractions, communicate calmly and adapt the environment. **LO:** 2, 8.
30. **Scenario:** A client becomes tired halfway through a meal. **Answer:** Pause safely, observe and document/report if new or repeated. **LO:** 5, 9.
31. **What should the caregiver NOT do?** Automatically feed a person who can eat independently with time. **Answer:** Support, not substitute. **LO:** 2, 8.
32. **MCQ:** Which is objective documentation? A) “Client was difficult” ; B) “Client refused half the meal and coughed twice after two sips” ; C) “Client is malnourished” ; D) “Family is careless.” **Answer:** B. **LO:** 9.
33. **Scenario:** A serious choking event is managed and the person appears better. What should happen? **Answer:** Follow medical review, incident reporting and handover requirements. **LO:** 6, 9–10.
34. **What should the caregiver NOT do?** Delay urgent help while waiting for routine supervisor approval. **Answer:** Activate the emergency response when life-threatening danger is present. **LO:** 10.
35. **Integrated scenario:** A client with a modified-fluid plan is drowsy, has taken little fluid, coughs after drinking and is offered an unlabelled snack. List the correct sequence. **Answer:** Know and check the plan; prepare safely; pause unsafe support; observe signs; do not serve the unlabelled snack or improvise texture/fluid; protect positioning and

dignity; escalate swallowing, alertness, intake and allergy risks; document facts, response and notifications; and follow up. **LO:** 1–10.

17. Answer key and explanations

Q	Answer	Professional reasoning	LO
1	B	Caregivers support, observe and report; they do not prescribe.	1
2	C	Repeated reduced intake is a pattern requiring assessment.	1, 4, 9
3	A	This is basic nutrition awareness, not a prescription.	1
4	Do not change	Therapeutic dietary changes require authorisation.	1, 3
5	False	Needs and preferences vary.	1–2
6	Offer authorised alternative	Choice and dignity can support intake.	2, 5
7	A	Mealtimes include dignity and independence.	2
8	Respect and report	Force is unsafe and undignified.	2, 5
9	Do not shame	Slow eating and assistance require respect.	2, 8
10	Support, not substitute	Independence should be preserved where safe.	2, 8
11	B	Signs require context and are not definitive.	3

12	Adapt access	Barriers should be addressed within role.	3
13	Document and escalate	Possible dehydration requires appropriate assessment.	3, 10
14	False	Individual targets are authorised instructions.	3
15	Notice, document, report	Warning signs do not equal diagnosis.	4
16	Do not prescribe	Supplements require appropriate guidance.	1, 4
17	A	Wet voice can indicate possible swallowing concern.	6
18	Stop and report	Continuing may be unsafe.	6–7
19	Do not give water automatically	Texture and aspiration risk require guidance.	6–7
20	False	One sign does not diagnose dysphagia.	6
21	Stop and escalate	Never improvise consistency.	7, 10
22	Do not modify	Follow authorised plan exactly.	7
23	B	“Keep clean” is one WHO Five Key.	8
24	Do not serve uncertain food	Temperature control and escalation matter.	8, 10
25	Do not serve until verified	Known allergy requires reliable information.	7, 10
26	False	Cross-contact can remain.	7

27	Emergency response	Breathing difficulty and swelling may be life-threatening.	7, 10
28	Adapt and prompt	Support accessible independence.	8
29	Reduce stimulation	Environment can affect participation and distress.	2, 8
30	Pause, observe, report	Fatigue may signal a new or relevant problem.	5, 9
31	Do not take over	Support, not substitute.	2, 8
32	B	It records observable facts and context.	9
33	Review and report	Choking requires follow-up even after apparent recovery.	6, 9–10
34	Do not delay	Life-threatening risk requires immediate response.	10
35	KNOW–CARE sequence	Integrates plan, safety, scope, documentation and escalation.	1–10

18. Recommended practical mealtime competency verification

Online completion does not prove feeding competence, choking-response competence, food preparation competence or real-world behavioural competence. Where an employer requires observed skill, NIC should offer or recommend separate verification.

Method	What it tests	Evidence
Mealtime simulation	Preparation, choice, positioning, pace and observation	Assessor checklist

Food and fluid observation exercise	Accurate intake and risk documentation	Marked record
Resistance/refusal scenario	Respectful communication and non-force response	Role-play rubric
Swallowing-awareness scenario	Recognition, stopping unsafe support and escalation	Structured decision record
Allergy and food-safety exercise	Checking labels, preventing cross-contact and storage decisions	Observation checklist
Choking first-aid course	Hands-on emergency response	Current recognised certificate
Supervisor observation	Real-world mealtime support and independence	Signed verification

High-level competency checklist

The learner should be able to:

1. check the current care plan, allergies, restrictions and texture instructions;
2. prepare a clean, safe and calm environment;
3. explain the meal and offer choice respectfully;
4. support independence and use authorised aids;
5. maintain appropriate positioning and attention;
6. notice reduced intake, fatigue, coughing, wet voice, food pocketing or distress;
7. avoid force, unsafe improvisation and unauthorised diet changes;
8. respond correctly to possible choking or allergic emergency through trained procedure;
9. document food, fluids, observations, responses and notifications;
10. report and escalate within the correct pathway.

A critical fail should include force-feeding, serving a known allergen without verification, changing a texture or fluid consistency independently, continuing despite signs of unsafe swallowing, serving uncertain contaminated food, delaying life-threatening emergency help or falsifying records.

19. Completion requirements

NIC should issue the credential when the learner has completed all 18 lessons, attempted all six module assessments, achieved at least **80% (28/35)** on the final assessment and acknowledged the scope statement. A failed final assessment may be retaken after review; NIC may permit up to three attempts before re-enrolment or supervisor review.

Where hands-on choking or feeding competence is required by an employer, the learner should complete separate recognised first-aid and practical verification. The online CPD demonstrates structured learning and scenario-based judgement; it does not prove safe physical feeding, dysphagia management, emergency manoeuvre competence, dietetic competence or clinical competence.

NIC should retain learner identity, enrolment and completion dates, curriculum version, lesson and module completion, final score and attempts, certificate ID, scope acknowledgement and practical-verification result where applicable. Records should be protected under NIC and employer privacy policies.

20. Certificate wording

Certificate title

NIC CPD Micro-Credential: Nutrition, Hydration & Mealtime Support for Professional Caregivers

Competency statement

This credential recognises successful completion of NIC continuing professional development in nutrition, hydration and mealtime support. The learner demonstrated knowledge-based and scenario-based understanding of person-centred eating and drinking, hydration awareness, malnutrition warning signs, appetite and refusal, independence, safe positioning, swallowing awareness, choking awareness, food safety, allergy and dietary-restriction checks, documentation and escalation within caregiver scope of practice.

The certificate should state the 5–6 CPD learning hours, completion date, assessment pass mark, certificate identifier and curriculum version. It must include: **“This CPD micro-credential is not a dietetics, nutrition, speech and language therapy, dysphagia, clinical or first-aid qualification. It does not authorise the holder to prescribe diets, diagnose swallowing disorders, independently modify food or fluid consistency, or act as a dietitian, nutritionist, therapist, clinician or dysphagia specialist.”**

21. Digital badge specification

Field	Recommended value
Badge name	NIC CPD Nutrition, Hydration & Mealtime Support
Description	Recognises completion of NIC CPD focused on safe, dignified and person-centred eating and drinking within caregiver scope.
Competencies represented	Nutrition awareness; hydration; mealtime dignity; independence; malnutrition-risk recognition; swallowing awareness; choking awareness; positioning; food safety; allergies; documentation; escalation
CPD hours	5–6 learning hours
Level	Professional CPD; intermediate workplace application
Evidence requirements	18 lessons, six module assessments, 35-question final assessment at 80% and scope acknowledgement; practical verification where required
Verification information	NIC verification URL or QR code, certificate ID, completion date and curriculum version
Limitation	Not a dietetic, clinical, dysphagia, speech-therapy or first-aid qualification
Review period	Recommend review at least every 24 months or sooner when guidance, local policy or emergency procedures change

22. NIC portal course description

Nutrition, Hydration & Mealtime Support for Professional Caregivers is a 5–6 hour NIC CPD micro-credential for caregivers and direct-care workers who already understand the fundamentals of caregiving. It develops the practical judgement needed to support safe, dignified and person-centred eating and drinking.

Learners practise supporting preferences and independence, recognising dehydration and malnutrition risk, responding to appetite loss and refusal, identifying possible swallowing concerns, following texture-modified plans, understanding choking response, applying

food-safety principles, checking allergies and restrictions, documenting meaningful observations and escalating concerns.

The course uses the memorable sequence **KNOW THE PERSON → PREPARE SAFELY → SUPPORT CHOICE & INDEPENDENCE → OBSERVE → RECOGNISE RISK → RESPOND WITHIN SCOPE → DOCUMENT → REPORT / ESCALATE**. It does not make learners dietitians, nutritionists, therapists, clinicians, dysphagia specialists or first-aid instructors.

23. Recommended downloadable job aids

A. Mealtime Observation Checklist

Check	Yes / No / N/A	Notes
Current care plan and restrictions checked		
Allergy information checked		
Texture/fluid instructions checked		
Person's preference and communication needs considered		
Seating and positioning safe		
Environment calm, clean and adequately lit		
Food and fluids accessible		
Independence supported		
Coughing, wet voice, fatigue or food pocketing observed		
Intake/refusal and response documented		
Concern reported or escalated where required		

B. Food & Fluid Observation Record

Recommended fields: client identifier; date; time; meal/drink offered; amount taken where policy requires; assistance provided; refusal; coughing or other signs; fatigue; pain or nausea reported; position; environment; response; outcome; person informed; follow-up required.

C. Mealtime Risk Checklist

Check for: wrong texture; wrong fluid; allergy uncertainty; expired or poorly stored food; unsafe water; unstable seating; drowsiness; repeated coughing; wet voice; food remaining in mouth; sudden appetite loss; repeated low intake; vomiting or diarrhoea; unplanned weight loss; choking; suspected allergic reaction; and missing authorised supplies.

D. Nutrition & Hydration Escalation Guide

Immediate emergency: choking with severe obstruction; breathing difficulty or collapse; severe allergic reaction; unresponsiveness; serious injury. Activate emergency response and follow trained procedure.

Prompt professional review: repeated coughing or wet voice; sudden inability to eat/drink; acute confusion or weakness; repeated vomiting/diarrhoea; severe dehydration concern; sudden appetite loss; repeated refusal; significant weight loss; wrong texture or unavailable prescribed fluid.

Routine review: gradual preference change, repeated small portions, equipment barrier, meal timing problem, environmental difficulty or recurring but non-urgent refusal.

The guide must include current employer, facility and local emergency contacts before portal publication. It must not imply one universal Nigerian reporting pathway.

24. References

[1] World Health Organization, Nutrition.

[2] World Health Organization, Food safety.

[3] National Institute for Health and Care Excellence, Nutrition support for adults: oral nutrition support, enteral tube feeding and parenteral nutrition, CG32.

[4] National Institute for Health and Care Excellence, Nutrition support in adults Quality Standard, QS24.

[5]: [https://www.clinicalnutritionjournal.com/article/S0261-5614\(22\)00034-6/fulltext](https://www.clinicalnutritionjournal.com/article/S0261-5614(22)00034-6/fulltext) "Volkert et al., ESPEN practical guideline: Clinical nutrition and hydration in geriatrics, Clinical Nutrition."

[5] World Health Organization, Five keys to safer food manual.

[6] American Red Cross, Adult & Child Choking: Symptoms and First Aid.

[7] National Institute for Health and Care Excellence, Dementia: assessment, management and support for people living with dementia and their carers.

[8] American Speech-Language-Hearing Association, Adult Dysphagia Practice Portal.

Source-use statement: WHO, NICE, ESPEN, the American Red Cross and ASHA materials are used as international or evidence-informed practice references. They do not constitute Nigerian law, NIC accreditation requirements or a substitute for local clinical, food-safety, emergency or employer policy. NIC should verify current sources and insert local procedures before publication.

25. Quality assurance review

Quality question	Finding	Implementation action
CPD value	The course focuses on judgement, risk recognition, safe support and escalation rather than diet memorisation.	Keep prerequisites visible and cross-link foundational courses.
Non-duplication	Basic caregiving is assumed; nutrition content is deliberately caregiver-level.	Avoid expanding into dietetics or clinical nutrition.
Person-centred care	Mealtimes are framed as dignity, culture, pleasure, social connection and independence.	Review examples with Nigerian caregiver educators.
Hydration	Reduced intake, risk factors and possible signs are covered with Observe–Record–Report.	Add local fluid documentation requirements.
Malnutrition	Warning signs and barriers are taught without diagnosis or unsupervised screening.	Map any approved local screening tool separately.
Swallowing safety	Warning signs, stopping unsafe support and escalation are covered.	Add employer-specific dysphagia pathway and specialist contacts.
Choking	Coughing versus severe obstruction and urgent help are covered without inventing a protocol.	Require separate current hands-on first-aid training where needed.

Food safety	WHO Five Keys and realistic home/facility risks are included.	Add current local food-safety procedures and outage guidance.
Scope	Diet prescription, texture changes, thickening, diagnosis and emergency competence are clearly limited.	Make scope acknowledgement mandatory.
Nigerian relevance	Local foods, family systems, heat, electricity, water and cultural/religious practices are acknowledged without stereotyping.	Conduct local contextual review before launch.
Assessment quality	28 of 35 final questions are scenario/application based.	Retain explanations and safety reasoning.
Practical value	Job aids and immediately usable mealtime steps are included.	Pilot with working caregivers and supervisors.
Professional credibility	Course includes competency framework, assessments, verification, certificate limits and review period.	Obtain formal NIC approval and employer consultation.
Evidence integrity	References are traceable and separated from Nigerian legal requirements.	Recheck links and guidance before each major revision.

Final implementation checklist

Before launch, NIC should confirm that the portal follows **COURSE → MODULES → LESSONS → MODULE ASSESSMENTS → FINAL ASSESSMENT → CERTIFICATE**. Each lesson should display objectives, core content, scenario, risks, key practice points and assessment. The portal should provide the four job aids and local escalation contacts.

Each employer should insert current contacts for the supervisor, clinical lead, dietitian or nutrition professional where available, swallowing specialist or speech and language therapist where available, emergency response, food-safety lead, allergy/emergency contact, complaints route and alternative escalation route. The course should be reviewed for accessibility, language, cultural fit, privacy, secure assessment handling and trauma-informed presentation.

The intended NIC professional standard is:

I understand that food and fluids are an important part of safe, dignified care. I can support a person to eat and drink safely and independently within my role, recognise warning signs, avoid unsafe improvisation, document meaningful changes and know when to seek appropriate help.